

Apollo Diagnostics

Comprehensive Cardiac Screening Report

Patient Demographics

Name: Ananya

Phone: 9625020380

Patient ID: AP-118492

Age/Sex: 55 / Female

Date of Birth: 14 August 1970

Life Stage: Postmenopausal

Report Date: 04 May 2026

Sample Collection: 03 May 2026 at 08:10 AM

Clinical Indication

Patient presented for preventive cardiac evaluation due to intermittent fatigue on exertion, occasional jaw heaviness, and family history of cardiovascular disease. Cardiac biomarker review, resting ECG, treadmill stress summary, and imaging support note requested.

Executive Summary

- Resting blood pressure is above the ideal preventive threshold.
- Total cholesterol is mildly elevated.
- Treadmill stress parameters show reduced reserve relative to age-adjusted expectation.
- Further clinician correlation is advised if symptoms persist despite non-alarming initial presentation.

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Vital Parameters and Baseline Cardiac Profile

Resting Blood Pressure: 128 mmHg (Above optimal)

Resting Pulse Rate: 78 bpm (Normal)

Body Mass Index: 27.4 kg/m² (Overweight)

Oxygen Saturation: 98% (Normal)

Biochemistry and Lipid Profile

Total Cholesterol: 210 mg/dl (Mildly elevated)

HDL Cholesterol: 46 mg/dl (Acceptable)

LDL Cholesterol: 136 mg/dl (Borderline high)

Triglycerides: 154 mg/dl (Borderline high)

Non-HDL Cholesterol: 164 mg/dl (Elevated)

Fasting Blood Sugar: Lower than 120 mg/ml (Screening category)

Resting ECG Summary

Rest ECG: Normal

Interpretation: Sinus rhythm maintained. No acute ST-T abnormality observed at rest. Correlate clinically with stress response findings.

Mild lipid elevation is noted. In women above 50 years of age, lipid changes should be correlated with symptom burden, menopausal status, and cardiovascular risk profile.

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Treadmill Stress Test Summary

Protocol Used: Bruce Protocol

Duration Completed: 6 minutes 42 seconds

Exercise Capacity: Moderately reduced for age

Max Heart Rate Achieved: 145 bpm

Predicted Max Heart Rate: 165 bpm

% of Predicted Max Achieved: 87.9%

ST Depression (Oldpeak): 1.2 mm

Exercise Induced Angina: No

ST Segment Slope: Flat

Test Termination Reason: Fatigue

No classical crushing chest pain reported. Patient described generalized tiredness and mild jaw discomfort at peak exercise. Recovery was uneventful. Clinical correlation is advised in the context of atypical symptom presentation.

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Imaging and Supportive Cardiac Risk Indicators

Vessels Colored by Fluoroscopy: Zero

Thalassemia Pattern: Fixed Defect

Chest Pain Type (History): Atypical angina

Symptom Intake Sheet

- Reduced exercise tolerance over the last 2 to 3 months
- Intermittent fatigue on stairs
- Occasional jaw tightness without severe chest pain
- No syncope
- No documented pedal edema

This combination of mild lipid elevation, exertional fatigue, atypical symptoms, and stress-test changes should be assessed in the context of sex-specific cardiovascular presentation. Normal resting ECG does not exclude ischemic risk in symptomatic patients.

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Consolidated Impression

Key Values for Clinical Review:

- Resting Blood Pressure: 128 mmHg
- Total Cholesterol: 210 mg/dl
- Fasting Blood Sugar: Lower than 120 mg/ml
- Rest ECG: Normal
- Max Heart Rate Achieved: 145 bpm
- ST Depression / Oldpeak: 1.2 mm
- Exercise Induced Angina: No
- ST Slope: Flat
- Vessels Colored by Fluoroscopy: Zero
- Thalassemia: Fixed Defect
- Chest Pain Type: Atypical angina

Clinical Advice

1. Review lipid abnormalities and exercise response with a physician.
2. If fatigue, jaw discomfort, breathlessness, or other symptoms persist, obtain cardiology consultation.
3. Consider correlation with sex-specific cardiac risk interpretation in symptomatic middle-aged women.
4. Lifestyle modification, repeat monitoring, and physician-guided follow-up are recommended.

This report is intended for physician review in conjunction with clinical findings, past medical history, and additional diagnostic evaluation where required. A laboratory report should not be used as the sole basis for diagnosis or treatment.

Authorized Signatory